

****DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 18

Age: 58 years old

Sex: Female

MRN: 000-TEST-18

Admission Date: [admission date]

Discharge Date: [discharge date]

Length of Stay: 4 days

****PRIMARY & SECONDARY DIAGNOSES****

PRIMARY:

- Acute exacerbation of Chronic Obstructive Pulmonary Disease (COPD), GOLD Stage IV (very severe) with FEV1/FVC ratio 38% predicted

SECONDARY:

- Systolic Heart Failure, EF 35-40% (Stage C, Class II)
- Obstructive Sleep Apnea (OSA), moderate severity on CPAP therapy
- Hypertension
- Type 2 Diabetes Mellitus

****HOSPITAL COURSE****

Patient 18 presented to Metro Medical Center via ED with acute dyspnea on exertion (DOE), increased sputum production, and oxygen desaturation. Initial SpO2 on room air was 82%, improved to 94% on supplemental O2 at 2L NC. Chest X-ray revealed hyperinflation without acute infiltrate. Patient was started on IV methylprednisolone 125 mg q.6h, nebulized albuterol/ipratropium combination q.4h, and IV fluids for hydration.

****Hospital Course Events:****

- Hospital Day 1-2: Aggressive bronchodilator therapy; IV steroids continued; chest PT performed
- Hospital Day 3: Transitioned to PO prednisone taper; SpO2 consistently 93-95% on 1L O2
- Day 4: Discharged on home O2 PRN with ambulation; baseline SpO2 92-94% on RA at rest

Pulmonology consult recommended optimization of inhaler regimen. Patient tolerated walking 100 feet without desaturation. Sleep medicine cleared for CPAP use; patient compliant at baseline.

****DISCHARGE MEDICATIONS****

| Medication | Dose | Frequency | Duration |

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| Tiotropium (LAMA) | 18 mcg | Once daily, inhaled | Ongoing |

| Vilanterol/Umecclidinium (LABA/LAMA) | 25/62 mcg | Once daily, inhaled | Ongoing |

| Fluticasone/Vilanterol (ICS/LABA) | 100/25 mcg | Twice daily, inhaled | Ongoing |

| Albuterol HFA | 90 mcg | 1-2 puffs q.4-6h PRN | Ongoing |

| Prednisone (TAPER) | 30 mg Day 1-3, 20 mg Day 4-6, 10 mg Day 7 | Once daily PO | 7 days |

| Furosemide | 40 mg | Once daily PO | Ongoing |

| Carvedilol | 12.5 mg | Twice daily PO | Ongoing |

| Atorvastatin | 80 mg | Once daily PO | Ongoing |

| Metformin | 1000 mg | Twice daily PO | Ongoing |

****Inhaler Technique:**** Patient instructed and demonstrated proper use with spacer device. Recommended air-drying of spacer after each use.

****FOLLOW-UP APPOINTMENTS****

1. Pulmonology clinic — in 2-3 weeks (Dr. [lastname], Metro Medical Center)
2. Primary Care — in 1 week for steroid taper assessment and labs
3. Cardiology — in 4 weeks for EF re-assessment and CHF optimization